

ENGLISH FOR WORK / SEPTEMBER 2026

Nursing and Allied Health English

Conversation lab

8 complete workplace dialogues, followed by eight additional role-play cases. Study the exchanges, then make the conversation your own.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Find the right language. Speak, write, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For nurses, charge nurses, physical therapists, occupational therapists, respiratory therapists, imaging staff, laboratory staff, care-team coordinators, and allied-health supervisors.

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

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Inside this conversation lab

Original fictional training conversations in professional English. These are realistic scripts, not transcripts of actual people. The professional roles are the speaker labels; all figures, incidents, and organizations are invented.

01 Shift handoff with a pending result

02 Escalating a change

03 Medication reconciliation discrepancy

04 Preparing teach-back

05 Interprofessional rounds

06 Correcting a chart entry

07 Family communication boundaries

08 A near-miss debrief

Then try eight independent role-play cases, followed by feedback criteria and references.

Read it. Hear it. Make it yours.

1. Read the setting. Identify the roles, the immediate problem, and what each speaker needs.
2. Read the whole conversation aloud with a partner. In a group, give a third person the observer role. For three-speaker scripts, assign each professional a separate voice.
3. Notice how the speakers ask a specific question, qualify a claim, disagree, explain a technical point, or agree on a next step. Underline language you would actually use.
4. Cover the script. Recreate the exchange using the situation and professional roles. Keep the meaning, but change the wording.
5. Switch roles and introduce a complication: a missing record, a different priority, an unavailable colleague, or a changed assumption. End with a clear outcome or unresolved question.

Register and terminology

These colleagues use concise professional language. In an internal technical meeting, familiar abbreviations can be natural. With a new colleague or a client, expand unfamiliar terms and check understanding. Keep disagreement directed at the evidence or proposal, and match the degree of certainty to what is known.

Independent study

Speak each role aloud. Pause before the reply and supply your own response before revealing the next line. Record a second version using invented details, then compare its clarity and tone with the script.

Professional context

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

FULL DIALOGUE 01 / 6 SPEAKING TURNS

Shift handoff with a pending result

Two nurses transfer care using the approved handoff process.

Roles: Outgoing nurse / Incoming nurse

Outgoing nurse: For the patient we've identified in the chart, the repeat lab result is still pending. The care team has been notified.

Incoming nurse: What prompted the repeat, and what follow-up has been requested?

Outgoing nurse: The initial sample was unsuitable. The record includes the collection time and the clinician's current plan.

Incoming nurse: I'll verify that plan in the chart. Who is checking for the new result?

Outgoing nurse: I'm handing that follow-up to you now. Please acknowledge it in our handoff record.

Incoming nurse: Understood. I'll check the result and escalate according to the documented plan and local procedure.

Notice the professional language

Discuss this wording: "What prompted the repeat, and what follow-up has been requested?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: note one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 02 / 6 SPEAKING TURNS

Escalating a change

A nurse reports a patient's change in condition.

Roles: Staff nurse / Charge nurse

Staff nurse: I'm concerned about a change from the patient's earlier assessment and need a prompt clinical review.

Charge nurse: Give me the situation and relevant background, then the findings you've observed.

Staff nurse: The patient is newly confused compared with this morning. I've recorded the current observations and followed our escalation process.

Charge nurse: Who has been contacted, and have they acknowledged the request?

Staff nurse: The responsible clinician has been contacted; I'm still waiting for acknowledgment.

Charge nurse: I'll support the escalation now under our local pathway. Keep the observations and response times clearly documented.

Notice the professional language

Discuss this wording: "Who has been contacted, and have they acknowledged the request?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: note one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 03 / 6 SPEAKING TURNS

Medication reconciliation discrepancy

A nurse and pharmacist clarify inconsistent records.

Roles: Ward nurse / Pharmacist

Ward nurse: The admission list and medication chart show different doses. I haven't treated either list as confirmed.

Pharmacist: What sources are available for reconciliation?

Ward nurse: The transfer document and the patient's reported home list. The prescriber still needs to resolve the discrepancy.

Pharmacist: I'll review those sources and contact the prescribing team through our process.

Ward nurse: The allergy entry is also incomplete. I'll flag that rather than leave it implicit.

Pharmacist: Good. We'll document the clarification and communicate the authorized plan before administration decisions proceed.

Notice the professional language

Discuss this wording: "What sources are available for reconciliation?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: note one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 04 / 6 SPEAKING TURNS

Preparing teach-back

A nurse and therapist coordinate discharge education.

Roles: Nurse / Occupational therapist

Nurse: The patient nodded during the equipment explanation, but we haven't checked their understanding.

Occupational therapist: Let's ask them to show how they would use it, with the approved instructions in front of them.

Nurse: I'll explain that we're checking how well we taught it, not testing them.

Occupational therapist: Use their preferred language and arrange a qualified interpreter if needed.

Nurse: If the demonstration shows a gap, we'll explain that part again and repeat the check.

Occupational therapist: I'll document the teaching, the observed response, and any further support required for discharge planning.

Notice the professional language

Discuss this wording: "The patient nodded during the equipment explanation, but we haven't checked their understanding." What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: note one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 05 / 6 SPEAKING TURNS

Interprofessional rounds

A therapist and discharge coordinator discuss readiness.

Roles: Physical therapist / Discharge coordinator

Physical therapist: Mobility assessment is complete, but the home access information is still missing.

Discharge coordinator: The family mentioned stairs. Can we record the patient as discharge-ready?

Physical therapist: My assessment addresses observed mobility here. It doesn't establish that the home setup is suitable.

Discharge coordinator: I'll obtain the access details and bring the outstanding issue to the team.

Physical therapist: I'll explain the functional findings and any assessment limitations at rounds.

Discharge coordinator: Then the responsible team can confirm the discharge plan with those factors in view.

Notice the professional language

Discuss this wording: "The family mentioned stairs. Can we record the patient as discharge-ready?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: note one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 06 / 6 SPEAKING TURNS

Correcting a chart entry

A nurse asks about an inaccurate documentation time.

Roles: Staff nurse / Charge nurse

Staff nurse: I entered the wrong observation time. The recorded value is correct, but the timestamp isn't.

Charge nurse: Use the approved correction process so the original entry and correction remain traceable.

Staff nurse: I shouldn't delete the entry and recreate it as though it were written earlier?

Charge nurse: Correct. State what is being corrected and follow the documentation policy.

Staff nurse: I'll also notify the clinician if the timing affects their interpretation.

Charge nurse: Yes. Accurate documentation and timely communication both matter; correcting one doesn't replace the other.

Notice the professional language

Discuss this wording: "I shouldn't delete the entry and recreate it as though it were written earlier?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: note one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 07 / 6 SPEAKING TURNS

Family communication boundaries

Two care-team members plan a response to a distressed relative.

Roles: Ward nurse / Patient liaison

Ward nurse: A caller wants detailed results, but I haven't verified their identity or permission to receive information.

Patient liaison: Acknowledge their concern and follow the consent and identity checks before any disclosure.

Ward nurse: They feel we're refusing to help. How can I explain the next step clearly?

Patient liaison: Say that you're checking how the team can share information appropriately, and identify who will follow up.

Ward nurse: I'll ask the responsible team to clarify the communication arrangement with the patient.

Patient liaison: I'll support the conversation and document the agreed contact route through the approved process.

Notice the professional language

Discuss this wording: "They feel we're refusing to help. How can I explain the next step clearly?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: note one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 08 / 6 SPEAKING TURNS

A near-miss debrief

A nurse and safety lead review a specimen-labeling near miss.

Roles: Nurse / Safety lead

Nurse: I noticed the label mismatch before the specimen left the ward. I followed the specimen procedure and reported it.

Safety lead: Let's walk through the sequence without assuming why it happened.

Nurse: Two requests were open at once. The label printed beside another patient's paperwork.

Safety lead: What made the mismatch visible, and which checks helped you catch it?

Nurse: The identity check. I'd like us to review how printouts are separated at the workstation.

Safety lead: We'll examine the workflow and record improvement actions without replacing the formal incident review.

Notice the professional language

Discuss this wording: "What made the mismatch visible, and which checks helped you catch it?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: note one natural professional expression and one place where the follow-up made the meaning clearer.

Eight more situations to make your own

The following cases are open role plays. They give you facts, contrasting roles, useful expressions, and a short model response. Use what you learned from the complete dialogues to create a longer exchange.

Preparation: two minutes. Conversation: three to five minutes. Feedback: one minute. Switch roles and repeat with the second-round challenge.

ROLE-PLAY CASE 01

Shift Handoffs and Clinical Prioritization

SHARED CASE

In a simulated handoff, the outgoing clinician has observations and background notes, but a requested result is still pending. The receiving clinician needs a clear transfer of information.

Role A | Responding professional

Give a handoff that the receiving person can confirm. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are taking over the work. Ask what remains open and repeat back the critical status or responsibility. Point out one detail that would be unsafe or misleading to assume.

Useful expressions

The current status is ...; the outstanding item is

The record shows

Please confirm who has accepted

Cover this until after your first attempt

The result is still pending. I'll separate the current situation, relevant background, my assessment, and the request in this handoff. Please repeat back the outstanding task so we can confirm understanding.

Second round

The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Write one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 02

Patient Assessment and Escalation

SHARED CASE

During a simulation, a patient reports feeling worse and the learner identifies a change from the earlier observation. The local escalation process is available.

Role A | Responding professional

Make a request with a clear action, purpose, and realistic timing. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You can help but need a clear request and its purpose. Ask what is required and whether the timing is fixed or negotiable.

Useful expressions

Could you provide ... so that ...?

Please confirm ... by [agreed time].

If that timing is not possible, please let me know

Cover this until after your first attempt

I'm concerned because this is a change from the earlier observation. I need an assessment through our escalation process now. Here is what I observed and when it changed.

Second round

The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Write one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 03

Medication Safety and Allergy Clarification

SHARED CASE

A fictional medication record and the patient's account contain different allergy information. The learner must communicate the discrepancy through the clinical process.

Role A | Responding professional

Clarify missing information before responding. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You need to know exactly what information is missing. Give one answer only after your partner asks a focused question; then ask them to confirm their understanding.

Useful expressions

When you say ..., do you mean ...?

Could you clarify which ...?

So, my understanding is Is that right?

Cover this until after your first attempt

The recorded allergy information differs from what the patient has told me. Could we verify the details through the appropriate clinical process? I will state the discrepancy clearly rather than assume either account is complete.

Second round

The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Write one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 04

Patient Education and Teach-Back

SHARED CASE

A patient in a role-play nods during an explanation of an approved discharge sheet. The learner needs to check whether the explanation was clear.

Role A | Responding professional

Explain a useful distinction in plain English. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are unfamiliar with one technical term in the case. Ask for a plain-English explanation and then paraphrase it. Do not pretend to understand a term you cannot explain.

Useful expressions

In this context, ... means

The difference is that

How would you explain that distinction to a colleague?

Cover this until after your first attempt

I want to check that I explained this clearly. In your own words, how will you follow the instructions when you get home? We can go over any part that is unclear.

Second round

Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Write one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 05

Interprofessional Rounds

SHARED CASE

In simulated rounds, one team member focuses on discharge timing and another raises an unresolved mobility concern.

Role A | Responding professional

Include relevant voices and clarify the decision process. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You hold a relevant concern that has not been discussed. Raise it when invited and ask who will decide. Help the facilitator state one unresolved question accurately.

Useful expressions

Let's hear ... before we decide.

The point we still need to resolve is

Who will make this decision, and what do they need?

Cover this until after your first attempt

Before we agree on timing, could we hear the mobility concern and the current assessment? Let's confirm the outstanding questions and who will address them in the care plan.

Second round

A participant interrupts or the meeting is running out of time. Protect a turn to speak and close with an accurate decision record.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Write one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 06

Documentation and Charting

SHARED CASE

A draft simulated note says a patient was "difficult." It contains no description of what was said or observed.

Role A | Responding professional

Separate observations, assumptions, and conclusions. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Useful expressions

The available evidence shows

This may indicate ..., but

We have not yet established whether

Cover this until after your first attempt

Let's replace that label with observable information. Record what the patient said or did, the relevant context, the action taken, and the response, following the documentation process.

Second round

Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Write one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 07

Difficult Families and Boundaries

SHARED CASE

A family member in a role-play says, "Nobody tells us anything." The learner can explain their role and arrange an appropriate conversation.

Role A | Responding professional

Acknowledge a communication problem and give a corrected message. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are frustrated because an earlier message created an expectation. Explain that expectation. Ask your partner to distinguish the correction from anything still uncertain.

Useful expressions

My earlier message did not make ... clear.

I'm sorry for The correct information is

Let me check that the revised explanation addresses

Cover this until after your first attempt

I can hear that the lack of information is frustrating. Let me explain what I can clarify and arrange contact with the appropriate team member for the questions outside my role.

Second round

The listener remains disappointed after the correction. Acknowledge the impact and restate the available next step calmly.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Write one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 08

Safety Events and Just Culture

SHARED CASE

A simulated safety report includes a fall and an assumption that a colleague was careless. The review has not established the contributing factors.

Role A | Responding professional

Separate observations, assumptions, and conclusions. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Useful expressions

The available evidence shows

This may indicate ..., but

We have not yet established whether

Cover this until after your first attempt

The report should describe the event and available observations. We have not established the contributing factors. I'll remove the unsupported judgment and use the reporting process to raise the concern.

Second round

Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Write one better follow-up question, then repeat the exchange.

Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and writes a 70-110 word message. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

Use the language responsibly

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

AHRQ: SBAR communication tool

<https://www.ahrq.gov/teamstepps-program/curriculum/communication/tools/sbar.html>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the next two prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Writing feedback begins with your draft. Vocabulary, grammar and review prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has eight practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

The two starter prompts use B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-06. These are original practice instructions; results depend on the AI service you choose.

Rehearse with an AI colleague

START PROMPT / COPY THROUGH END PROMPT

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Nursing and Allied Health English

Professional audience: nurses, charge nurses, physical therapists, occupational therapists, respiratory therapists, imaging staff, laboratory staff, care-team coordinators, and allied-health supervisors

Scope: Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Dialogue 01: Shift handoff with a pending result

Original fictional setting: Two nurses transfer care using the approved handoff process.

Professional roles: Outgoing nurse / Incoming nurse

Published dialogue (reference script, not my own performance):

Outgoing nurse: For the patient we've identified in the chart, the repeat lab result is still pending. The care team has been notified.

Incoming nurse: What prompted the repeat, and what follow-up has been requested?

Outgoing nurse: The initial sample was unsuitable. The record includes the collection time and the clinician's current plan.

Incoming nurse: I'll verify that plan in the chart. Who is checking for the new result?

Outgoing nurse: I'm handing that follow-up to you now. Please acknowledge it in our handoff record.

Incoming nurse: Understood. I'll check the result and escalate according to the documented plan and local procedure.

END REFERENCE

TASK: Interactive workplace role-play

1. Use the supplied case or dialogue as the starting situation. Give a two-sentence briefing and offer two relevant professional roles for me to choose from. For a supplied dialogue, use its actual roles. Ask which role I want, then stop and wait. Do not write my replies.

2. After I choose, identify who you will play and the immediate communication goal. Play the other professional; where a meeting requires a third person, label each of your speakers clearly. Start with one natural workplace turn and wait for my reply. Keep most turns to one to three sentences and ask no more than one question at a time.
3. Let the exchange develop across six to ten learner turns, or end earlier if I type "feedback". Respond to what I actually say. Include a plausible clarification, disagreement or tradeoff without silently changing the starting facts. Mark any added constraint as a fictional second-round variation. Do not resolve approvals, evidence or commitments that remain uncertain.
4. Stay in role during the exchange. If my meaning is unclear, ask for clarification naturally. Give a hint only if I ask or cannot proceed. Do not deliver a model conversation in advance.
5. At the debrief, quote two of my phrases: one successful choice and one worth improving. Check factual accuracy, language, register and whether the next step was clear. Give up to three focused suggestions. Ask me to retry the weakest turn; wait. Only then offer an alternative wording and a harder replay.

END PROMPT

Copy this prompt online or choose another lesson and support level

Create more scenario dialogues

START PROMPT / COPY THROUGH END PROMPT

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

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Incoming nurse: What prompted the repeat, and what follow-up has been requested?

Outgoing nurse: The initial sample was unsuitable. The record includes the collection time and the clinician's current plan.

Incoming nurse: I'll verify that plan in the chart. Who is checking for the new result?

Outgoing nurse: I'm handing that follow-up to you now. Please acknowledge it in our handoff record.

Incoming nurse: Understood. I'll check the result and escalate according to the documented plan and local procedure.

END REFERENCE

TASK: Extend the conversation library

1. Propose six distinct, common scenarios in this field beyond the supplied case or script. For each, name the setting, two or more professional roles, the communication problem and one useful terminology focus.

Include routine coordination, clarification, a complication, disagreement, handoff and follow-up where relevant. Choose scenarios that fit this profession, rather than forcing unsuitable situations into it.

2. Ask me to choose one scenario, or request all six in sequence. Stop and wait. Do not write all the scripts before I choose.

3. For the selected scenario, write an original fictional dialogue of 12-18 substantial speaking turns between two or three professionals. Name each role, establish an actual work problem, and let the exchange progress through questions, clarification, competing constraints and a credible next step or explicitly unresolved issue. Use the occupation's natural nomenclature and register. Avoid an interview between a teacher and a learner, generic small talk, and inserting a glossary definition into every reply. Explain specialist terms outside the dialogue instead.

4. After the script, explain five useful expressions in context, identify two grammar or register choices and ask three questions about the speakers' reasoning. Withhold the answers until I attempt them. Add one role-switch challenge.

5. Before presenting a script, check names, numbers, chronology, roles and terminology for consistency. Label invented facts and acknowledge uncertain specialist usage. If I requested all six, deliver one complete script at a time and wait for "next". Make each scenario materially different.

END PROMPT

Copy this prompt online or choose another lesson and support level